

YOUR CONDITION · TENDRA

The most distinctively-located pain in the body.

What the SI joint is, why it gets missed by almost everyone except a clinician who is looking for it, and what actually helps.

A short read · About 3 minutes · **For anyone who can point to exactly where it hurts**

There is a spot, roughly the size of a fifty-cent piece, sitting just above and to the side of the tailbone. When it hurts, it hurts there. When it is the source of a person's pain, they almost always point to that exact spot. And yet most patients have never heard the name of the joint underneath it.

This article is about that joint.

01 Where it is, and why it gets missed

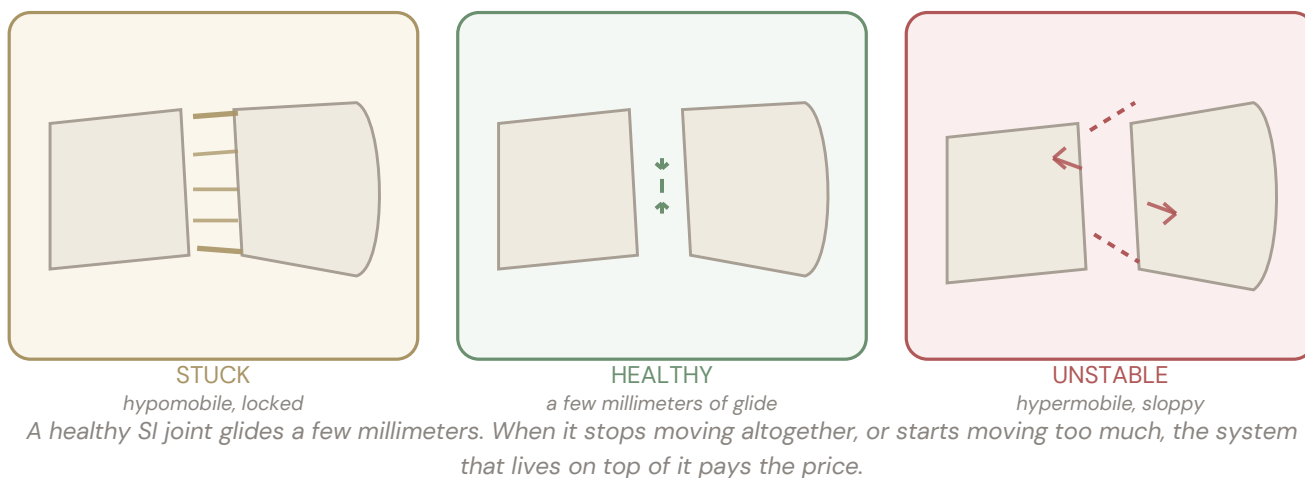
The sacroiliac joint — usually shortened to "SI joint" — is the seam where the sacrum meets the pelvis. You have one on each side. They are the structural handoff between your spine and your legs. Every step you take, every load your body carries, gets transmitted through these two joints.

Despite that workload, they barely move. A healthy SI joint glides only a few millimeters in any direction, and that is by design. It is not a moving joint in the way a hip or shoulder is; it is a load-distributing joint that gives just enough to absorb shock without giving so much that the pelvis falls apart.

That tiny range of motion is part of why SI joint problems get missed. Imaging shows almost nothing useful here — the motion is too small to capture in a snapshot. The diagnosis is clinical. SI joint pain also has a habit of impersonating other things: it can radiate down the back of the thigh and feel like sciatica, sit close enough to the hip that patients call it hip pain, or blur into generic low back pain. The giveaway is usually simple: a single finger landing just above and to the side of the tailbone.

02 Two ways it goes wrong

The phrase "SI joint dysfunction" gets used loosely. There are actually two different problems hiding under that name, and they need different care.



Stuck (hypomobile). The joint has stopped gliding. The two bone surfaces are locked in one position. The body interprets this as a problem — surrounding muscles tighten around the joint as protection, and the area becomes sore and stiff. This is the most common version, and the one chiropractic adjustments are particularly good at addressing.

Unstable (hypermobile). The opposite problem. The joint moves too much. The ligaments that normally hold it snug have become loose, and the joint shifts around with movement and loading. This is common during and after pregnancy, and in people with naturally loose joints. Adjustment alone does not fix this — strength and stability work is the answer.

It matters which one you have. A stuck joint that gets stretched and a loose joint that gets adjusted both feel worse, not better. This is why a real exam matters more than a generic protocol.

IN PLAIN TERMS

If you can point to your pain with one finger, and that finger lands just above and to the side of the tailbone — there is a real chance the SI joint is the source, regardless of what an MRI says.

03 What actually helps

Stop sitting unevenly. Wallet under one buttock, always crossing the same leg, leaning on one elbow at the desk — these habits add up to thousands of hours of asymmetric load on the SI joints. The fix is mostly just noticing.

Hip mobility matters. The hips and the SI joints share work. When the hips are tight, the SI joints get forced to compensate. Most SI flare-ups improve dramatically when hip mobility improves.

Glute strength is non-negotiable. The gluteal muscles are the main stabilizers of the pelvis. When they are weak, the SI ligaments end up doing work they were not designed for, and the joint either gets stuck or sloppy.

For the stuck type: get adjusted. Manual restoration of joint motion is what chiropractic was built for, and the SI joint responds particularly well.

For the unstable type: do not stretch it. With a hypermobile joint, that makes things worse. Stability work — single-leg balance, glute strength, core integration — is the medicine. Be patient.

A stuck SI joint that gets stretched, and a loose SI joint that gets adjusted, both feel worse. A real exam matters more than a generic protocol.

04 When to call

CALL THE OFFICE OR SEEK CARE IF YOU NOTICE

- **Numbness in the groin or saddle area**, or new trouble controlling the bladder or bowels.
- **Progressive weakness in a leg** — actually losing strength.
- **Fever along with SI joint pain** — uncommon but a same-day call.
- **Pain that follows a real injury** — a fall on the buttocks, a car accident — and is not improving.
- **Severe pain at night** that is unrelated to position.

For everything else: notice how you sit, work the hips and glutes, get adjusted if you are the stuck type, train the stabilizers if you are the unstable type, and trust the process.

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About this guide. This is general patient education and is not medical advice for any specific person. SI joint presentations vary considerably between individuals. Any concerns about a specific symptom should go directly to the treating chiropractor or a relevant medical provider. If symptoms feel like a medical emergency, call 911 or go to the nearest emergency department.